

RHEA JOEL DCUNHA

rheajd@bu.edu | +1 617-212-8132 | [LinkedIn](#) | [Portfolio](#)

EDUCATION

Boston University Master of Public Health - Epidemiology and Biostatistics GPA 3.92/4.0 Clinical Trials, Cancer Epidemiology, Financial Management for Health Programs, Global Mental Health	May 2026
MGM Dental College & Hospital Bachelor of Dental Surgery GPA 4.0/4.0 General Human Physiology & Biochemistry, General Pathology & Microbiology, Oral and Maxillofacial Surgery	Oct 2023

TECHNICAL SKILLS AND CERTIFICATIONS

Programming & Data Analysis:	R, SAS, IBM SPSS, Python, VBA, SQL
Research Tools:	NVivo, Dedoose, Qualtrics, Covidence, REDCap, EHR, Zotero
GIS & Mapping:	QGIS, ArcGIS
Data Visualization & Management:	Microsoft Power BI, Microsoft Excel, Microsoft Access, Canva, WordPress
Certifications:	CITI Program: Medical Campus Biomedical & Sociobehavioral Researchers, HIPAA & Research Data Security, Information Privacy & Security, GCP

RESEARCH & CLINICAL EXPERIENCE

Research Fellow , <i>Boston University School of Public Health</i>	July 2026 – Present
- Supported \$29M NIH-funded project focused on improving causal inference in ADRD prevention research by conducting systematic literature reviews, risk-of-bias assessments, critical appraisal of epidemiologic evidence on modifiable dementia risk factors. - Applied epidemiologic methods to evaluate study validity and causal relationships in dementia research, supporting rigorous evidence generation for prevention strategies targeting Alzheimer's disease and related cognitive outcomes.	
Research Project Coordinator , <i>SPHERE Lab</i>	July 2026 – Present
- Managed financial operations for a \$20M+ portfolio of 5 NIH and foundation-funded grants (R01, RF1, P01), reconciling SAP reports, monitoring expenditures, and forecasting budgets to ensure compliance and optimize resource allocation. - Led regulatory and grant administration workflows across NIH-funded studies, coordinating IRB amendments, DUAs, RPPR documentation, contracts, and hiring processes to maintain research continuity and compliance.	
Research Assistant , <i>TIME-AD</i>	June 2025 - May 2026
- Identified evidence that hearing aids reduce dementia risk by conducting a systematic review in Covidence; performed literature searches, extracted, cleaned and validated data using R, including QA/QC checks, and conducted meta-analyses - Developed and tested gold-standard survey questions in Qualtrics to assess dementia-related social determinants; supported research dissemination via abstract submission to SER (Society for Epidemiologic Research)	
Graduate Research Assistant , <i>Jacobson Lopez Research Group</i>	Jan 2025 - May 2026
- Led trauma-informed research on healthcare providers' care of marginalized sexual assault survivors by recruiting 20+ participants, managing REDCap data, and shadowing interviews, ensuring high-quality data collection - Conducted targeted literature reviews and performed qualitative coding and thematic analysis using Dedoose; executed quantitative statistical analyses in IBM SPSS to identify healthcare service gaps; and submitted abstracts to APHA and SSWR	
Graduate Administrative Assistant , <i>Boston University School of Public Health</i>	Jan 2026 - May 2026
- Built VBA automation tools to streamline NIH biosketch formatting and standardize sections, fonts, and page limits; assisted in drafting and submitting IRB- and sponsor-compliant research and grant proposals; reducing preparation time by 65% - Automated export of publications by building a Python-based pipeline, saving 20+ hours of manual work	
Clinical Documentation Specialist , <i>IKS Health</i>	Nov 2023 - Jul 2024
- Served as Lead Clinical Documentation Specialist for a newly onboarded psychiatric team, managed EHR documentation for 80+ patients weekly to ensure accuracy, consistency, and high-quality clinical records - Trained and supervised 10 scribes in psychiatric documentation using EPIC, improved workflow efficiency, reduced errors, and supported seamless patient care continuity	

PUBLICATIONS

- D'cunha R, Agrawal A, Kolge N, Ravindranath V, Rodrigues L, Umalkar D (2023). Evaluation of the Knowledge, Awareness and Attitude Towards Orthodontic Treatment amongst Adult Population in Navi Mumbai: A Cross-Sectional Questionnaire based Survey. *Scholars Journal of Dental Sciences*. 11(08), 171–181
- D'cunha R, Kolge N, Rodrigues L, Agrawal M, Minase D, Jamenis S, Madhyan D, Ravindranath V (2023). Long Term Comparative Assessment of Nutrient Intake in Adolescent Urban Indian population undergoing Orthodontic Treatment. *European Chemical Bulletin*. 12(3), 7343–7356

PROJECTS

Association Between SES and Access to Improved Water Sources in Rural Zambia (<i>UNICEF WASH Dataset, R/RStudio</i>)
Performed cross-sectional analysis on data (n=1,170) using prevalence differences, chi-square testing, and multivariable logistic regression, estimating an adjusted OR = 2.07 (95% CI: 1.62–2.64) for improved water access among higher-wealth households
Association Between Household Wealth and Child Growth Outcomes in Zambia (<i>2018 DHS Dataset, SAS</i>)
Conducted cross-sectional analysis (n=8,744) using ANOVA, multiple linear regression, and multivariable logistic regression, estimating adjusted OR = 1.251 (95% CI: 1.081–1.448) for severe stunting among poorer households and $\beta = -0.147$ (95% CI: -0.244, -0.051) lower height-for-age Z-scores compared to middle-wealth households
CDC Tuberculosis Surveillance Dashboard: US Trends and Disparities (2000-2023) (<i>Excel</i>)
Designed interactive dashboard visualizing 24-year TB surveillance data using pivot tables, VLOOKUP functions, and conditional formatting to display 5 KPIs across demographic subgroups for CDC stakeholders and public health partners
Spatial Distribution and Socioeconomic Patterns of Rodent Complaints in Boston (<i>QGIS, ArcGIS, City of Boston 311 data</i>)
Conducted geospatial analysis integrating 311 complaint data, ACS census tract income estimates, and MBTA transit locations using QGIS geoprocessing tools (spatial joins, buffer analysis, attribute management) to identify rodent activity hotspots concentrated in central Boston neighborhoods with significant clustering within 500m of transit stations